

## FORESKIN CONDITIONS GUIDANCE

Consider Urology Email Advice & Guidance

### BALANITIS

Clean penis daily with lukewarm water and dry gently  
Do not attempt to retract if fixed; do not use soap, bubble bath or baby wipes  
Use 1% hydrocortisone cream for 2 weeks

Non-specific dermatitis / candida  
Add imidazole cream / single dose 150mg PO fluconazole in adults

Irritant / contact allergic dermatitis  
Stop suspected triggers

Bacterial  
PO flucloxacillin or clarithromycin 1week

In adults: for Gardnerella / anaerobic infections

PO metronidazole 400mg BD 1 week  
Consider referral STI clinic / swabs / HIV testing

? severe / persistent / recurrent balanitis  
check HbA1c

### PHIMOSIS ([baus guidance](#))

**PHYSIOLOGICAL:**  
Non-retractile healthy foreskin

**PATHOLOGICAL:**  
Symptomatic white  
Scarring of foreskin tip  
([paeds guidance](#))

No intervention (physiological adherence of foreskin to glans normal in children up to at least 10 years)

Lichen sclerosus or BXO

Persistent significant symptoms as adult

Consider referral to UROLOGY for circumcision  
[CCG funding criteria](#)

### PARAPHIMOSIS

Gentle compression with saline-soaked swab and reduction

recurrent

unsuccessful

**EMERGENCY UROLOGY REFERRAL**